

**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,
Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,
James Zappia james.zappia@art.com.au

Sent: Wednesday 29 July at 16:10

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) | 

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2744](tel:0735422744)
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @[Solv.com.au](#) email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not

miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymangement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_injurymangement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -
incorrect application of EB12, Award and QH policies - ECC
3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO
NOT click on any links or open attachments unless you
recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard
management correspondence between 2 and 15 July
2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl

7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.
- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

- 4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
- 5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
- 6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
- 7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
- 8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

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